February 23, 2026

The Honorable Thomas Keane, M.D.
Assistant Secretary for Technology Policy,
National Coordinator for Health Information Technology
Department of Health and Human Services
200 Independence Avenue, S.W.
Washington, DC 20201

Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane:

AARP, which advocates on behalf of 125 million Americans age 50 and older, thanks you for seeking information on incorporating artificial intelligence (AI) into clinical health care. We appreciate the Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology's interest in harnessing technology to improve patient experience and outcomes. Furthermore, we share your belief that – when used appropriately – technology and AI can lower health care costs through increased efficiency and productivity. Our focus is on ensuring older Americans and family caregivers are confident their privacy is protected, their data is secure, and a human relationship with their provider is maintained. Above all, a human must be in the loop for all consequential decisions – and nearly all health care decisions are consequential. If older Americans are to entrust more of their health care to an AI-enabled clinical tool, then that technology must be worthy of their trust.

Trustworthiness necessitates that fairness, transparency, and accountability guide all uses of AI or other algorithmic tools used to make consequential decisions regarding a patient's health, coverage, or well-being. Patients must feel secure that the AI tools being used to make potentially life and death decisions are:

- Fair, reliable, accurate, and incorporate patient preferences and goals, and not result in unjustifiable limits to consumer's access to care;
- Transparent, so that when a consumer, or provider involved in a consumer's care, interacts with such a tool, they have a clear understanding of the benefits and limitations of the tool and any results it generates; and
- Accountable, such that patients who have been adversely affected by a decision informed by an algorithmic tool have access to a fair and meaningful process to challenge those decisions and their outcomes.

ASTP/ONC ask several questions regarding AI regulation, reimbursement, and research & development. We are limiting our responses to specific questions with a direct consumer impact.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

The Health Insurance Portability and Accountability Act's (HIPAA) privacy and security protections remain foundational to safeguarding personal health information (PHI) and to ensuring that individuals maintain meaningful control over sensitive information about their health. Yet since HIPAA's enactment, the sources, types, and volume of health and health-related data have expanded dramatically. AARP supports updating HIPAA and related federal policy to keep pace with technological innovation, to strengthen trust, and to enable person- and family-centered care.

A critical starting point is clarifying the definitions of "covered entity" and "business associate" at 45 CFR 160.103 to expressly include AI tools and platforms such as electronic health records, health apps and web portals. The web of data is becoming increasingly tangled as machine learning tools pull information from various sets and sources. Legal accountability and responsibility must be clear, and a chain of custody must be followed no matter where the data goes.

Second, there should be rigorous enforcement of HIPAA's de-identification standards. De-identified health and health-related information is not protected by HIPAA, which heightens the importance of getting de-identification right. In the case of AI platforms, the *Expert Determination* method is impractical and insufficient for de-identifying PHI. Studies have already indicated that AI tools are capable of re-identifying health data. We urge the HHS Office for Civil Rights to hold covered entities accountable for fully complying with the *Safe Harbor* method of de-identification by removing all 18 identifying elements prior to any disclosure or external use. These elements include names; geographic subdivisions smaller than a state; all elements of dates (except year) directly related to an individual; telephone and fax numbers; email addresses; Social Security numbers; medical record numbers; health plan beneficiary numbers; account numbers; certificate/license numbers; vehicle identifiers; device identifiers; web URLs; IP addresses; biometric identifiers; full-face photographs and comparable images; and any other unique identifying number, characteristic, or code. Strong, consistent enforcement of *Safe Harbor* will help ensure that de-identified data truly minimizes re-identification risk.

At the same time, federal policy must address the widening gap between HIPAA's covered entities and the rapidly growing universe of consumer-facing AI health technologies. Today, many direct-to-consumer applications and services are marketed as wellness, fitness, or convenience tools. These direct-to-consumer products collect, track, or infer sensitive health and health-related information without being subject to HIPAA. They, in turn, can sell that health data to other third-party entities, which are also not covered by HIPAA. Both the consumer-product developers and the third-party data entities are typically governed only by their own terms of service and privacy policies, which can be lengthy, difficult to understand, and subject to unilateral change. As commercial entities expand their roles in collecting, managing, and refining data—and in some cases begin to offer clinical insights—stronger, enforceable consumer protections and accountability standards are needed to sustain trust and promote responsible innovation.

To this end, ASTP/ONC should also coordinate closely with the Federal Trade Commission and, where appropriate, the Food and Drug Administration. The FTC's enforcement authority is limited to when companies violate their own stated privacy commitments. Although the FTC's authority to address unfair and deceptive practices is essential, there should be enforceable rules regarding unreasonable or harmful data use. ASTP/ONC, CMS, the FTC, and FDA should work together to develop clear, consumer-friendly standards for privacy notices and terms, to promote consistent expectations for third-party data practices and to protect both the security of the data and the safety and privacy of consumers.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

AARP has done significant research into consumer sentiments regarding AI. In particular, we've looked at patterns of AI use, expectations of the role that AI does and will play in health care, and concerns toward integration of AI into health technology that linger among older Americans. We also note that family caregivers are increasingly using technology, such as remote monitoring tools, health tracking software, and assistive devices. Technology helps family caregivers be organized as they juggle their many responsibilities and oversee or monitor their loved one's care, even when they cannot be physically present.

As older Americans look toward the future of health care, many see artificial intelligence as a tool that could ease some of the everyday challenges that come with managing their well-being. The AARP *Navigating the World of AI* report shows that older Americans are especially hopeful that AI might step in to help with the countless small but essential tasks that make up daily health management—things like remembering medications, organizing health information, tracking symptoms, or sorting through complicated instructions from various providers. These "micro tasks" often fall on patients or family caregivers, and Americans recognize just how much time, attention, and energy these responsibilities demand. They imagine AI serving as an assistant that helps keep things on track, reduces errors, and ultimately supports their ability to remain independent.

They also see potential in AI's ability to monitor health continuously and reliably. With a majority expressing interest in AI-enabled monitoring and diagnostics, many Americans hope these tools could help them detect problems earlier, stay aware of changing symptoms, and make more informed decisions about when to seek care. In their view, AI could help simplify complex medical information and offer personalized guidance, making health care feel more navigable and less overwhelming. And because medication management is such a persistent challenge—particularly for those balancing multiple prescriptions—they see value in AI's ability to provide reminders or support adherence routines. These possibilities are attractive because they directly address the areas where patients and family caregivers often feel stretched thin.

Yet even as they recognize these potential benefits, older adults approach the idea of AI in clinical care with noticeable caution. Their worries begin with a fear that AI could replace the human qualities they value most in health care—empathy, conversation, reassurance, and the ability to understand nuance. They express low enthusiasm for AI stepping into roles they consider "human powered," such as therapy or emotional support, believing that these are areas where the human touch is not only beneficial but necessary. This concern about reduced human interaction reflects a broader anxiety about what might be lost if AI overtakes clinical environments. A human must remain in the loop to ensure accountability and trust.

Another prominent worry centers on accuracy. Older adults in particular are not convinced that AI will interpret symptoms correctly or make sound clinical judgments, especially in complex situations. Their ambivalence is captured in the report's finding that they are split evenly on whether AI's benefits outweigh its risks—a sign that concerns about safety, reliability, and unintended consequences are deep-seated. They also fear that AI may oversimplify medical problems that require context, intuition, and careful listening. For many people, real-world clinical care is full of subtleties: symptoms that don't fit a pattern, multiple chronic conditions that interact unpredictably, or personal histories that shape health decisions. They question whether AI can truly account for these complexities.

Taken together, the report paints a picture of older Americans who are genuinely open to the promise of AI in clinical care—but only if it reinforces, rather than replaces, the human centered care they value. They want AI to relieve burdens, simplify complexity, and help them stay healthy and independent, while still preserving the connection, understanding, and judgment that come only from human clinicians. AI may have a role to play, they believe, but it must be carefully shaped, thoughtfully integrated, and always aligned with the realities of the patients and family caregivers whose lives it aims to improve.

Conclusion

AARP appreciates the opportunity to respond to your request for information on Artificial Intelligence in the delivery of clinical care. We understand the challenges of balancing privacy and security with the desire to improve care through data and technology. As you lay the groundwork for future innovation, we urge you to ensure people can retain control of the use of their own information and that individual and family caregiver needs will be given the utmost priority. We look forward to working with you to make sure that the adoption of AI into clinical care is fair, accurate, transparent, and supportive of needs of older adults. If you have any questions, feel free to contact me, or have your staff contact Andrew Scholnick of our Government Affairs team at ascholnick@aarp.org.

Sincerely,

Meg O'Reilly

Megan O'Reilly
Vice President
Government Affairs, AARP